THE RELATIONSHIP BETWEEN PEDAGOGICAL APPROACHES ON CLINICAL SKILLS OF NURSING STUDENTS AT POLYTECHNIC COLLEGE OF DAVAO DEL SUR

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CHAPTER I

INTRODUCTION

Background of the Study

Pedagogical approaches play a critical role in shaping the clinical competence of nursing students. Effective teaching methods ensure that students acquire both theoretical knowledge and clinical skills necessary for safe, high-quality patient care. At Polytechnic College of Davao Del Sur (PCDS), pedagogical approaches primarily include instructor-led demonstrations, return demonstrations using low-fidelity mannequins, and peer-assisted learning. These approaches enable students to rehearse and refine procedural techniques in a controlled environment before encountering real patients. The present correlational study explores how variations in the type, exposure and quality of these pedagogical approaches relate to objective clinical skill outcomes measured via OSCE scores and standardized error checklists among nursing students at PCDS.

Despite increased attention to simulation and active learning globally, many nursing schools especially in resource-constrained settings rely on low-fidelity mannequins and return demonstrations for clinical skills training. At PCDS, the absence of high-fidelity simulation (HFS) and variability in supervision, practice hours, and feedback protocols raise concerns about the consistency of clinical skill acquisition and error rates during summative assessments and clinical exposures. Without empirical, context-specific evidence, faculty lack guidance on which pedagogical elements are most strongly associated with better OSCE performance and fewer observable errors. This gap complicates curriculum decisions and resource prioritization aimed at improving graduate

readiness and patient safety in Digos City. Knowing these etiologies will serve as a basis for creating effective, evidence-based interventions.

Landmark meta-analyses and International research show that simulation-based education and active, feedback-rich pedagogies improve psychomotor skills, clinical reasoning, and safety-related behaviors among nursing students (Cant & Cooper, 2020; Cook et al., 2013; Paige et al., 2024). Systematic reviews indicate that both high- and low-fidelity simulations can produce meaningful gains, and that design elements such as structured debriefing, deliberate practice, and immediate feedback often explain more variance in outcomes than fidelity alone (Frangi et al., 2024; McGaghie et al., 2021). Global simulation guidelines emphasize quality standards for replacing clinical hours with simulation, reinforcing that well-designed low-cost simulations may be appropriate when fidelity is matched to learning objectives (NCSBN, 2014).

While international literature provides a robust foundation, in the Philippines, nursing programs have adapted to blended and flexible learning models, particularly since the COVID-19 pandemic, expanding the use of simulation modalities and digital resources (Garcia & dela Cruz, 2023). CHED memoranda and local curricular policies support outcomes-based education and recognize simulation practice as vital components of Related Learning Experiences (RLE). Provincial schools, including in Mindanao, report limited access to HFS and rely on mannequins, peer learning, and faculty-led demonstrations; local studies and reports suggest that low-fidelity simulation coupled with structured feedback can improve student performance but empirical, institution-level correlational data remain sparse.

Although international literature supports simulation and active pedagogies, there is limited correlational research linking specific pedagogical exposures within low-resource nursing schools such as PCDS to objective clinical performance measures like OSCE scores and standardized error counts. Therefore, this study directly addresses that gap by rigorously quantifying the association between measurable pedagogical variables and objective clinical outcomes among PCDS nursing students, thereby providing the context-appropriate, empirical evidence necessary for informing curriculum improvements and optimizing resource allocation within similar low-resource academic settings.

Review of Related Literature

In this section, related literature such as scholarly articles, books and other sources relevant to the study are moreover reviewed for further understanding of the study.

Lecture - Based Learning

Lecture-based learning has long been the most widely used pedagogical approach in nursing education, serving as a foundation for theoretical instruction. However , while lectures effectively transmit large amounts of content, they often fall short in developing higher-order thinking skills in nursing students (Wei et al., 2023). Moreover, studies have shown that excessive reliance on lectures may reduce student engagement and motivation, limiting opportunities for active participation (Yoon et al., 2024). This is

particularly raising concerns given the complex nature of modern healthcare which demands advanced cognitive abilities to ensure safe and effective patient care.

Several studies have concluded that lectures, while providing structured content delivery, lack opportunities for students to integrate practice and reflection. For instance, Alharbi et. al (2024) found that while lectures are effective for cognitive learning , they are inadequate for developing clinical reasoning.

Similarly, Li et al. (2022) emphasized that although lectures provided a baseline knowledge, this restricted students' abilities to translate concepts into clinical judgement and critical thinking skills necessary for patient care. To address these limitations, lecture-based learning needs supplementation with active strategies that enhance psychomotor development and practical competence (Nurfianti and Mullen, 2024). Consequently, to bridge the gap between theoretical knowledge and practical application, studies have explored alternative pedagogical strategies, including high-fidelity simulations, that move beyond passive content transmission to prioritize a student-centered engagement.

Simulation-Based Education and Fidelity

In recent years, simulation-based learning, particularly through the use of low- and high-fidelity mannequins, has emerged as a key pedagogical approach in nursing education (Mishra & Trivedi, 2023). Its effectiveness in enhancing students' knowledge, clinical reasoning, and psychomotor skills surpasses that of lecture-based instruction alone. Simulation allows students to work in a hospital-like atmosphere and get

healthcare and nursing experience before beginning their careers (Koukourikos et al., 2021). A key benefit of this model is the creation of a sense of security. Students can develop knowledge, skills, and confidence in a safe environment without risks to patients, a crucial advantage for the learners as future healthcare professionals. Reinforcing this application, a study by Tong et al. (2024) has demonstrated that combining high-fidelity simulation with digital tools improved critical thinking and clinical competence.

The concept of fidelity, the degree to which a simulation replicates reality, is central to this pedagogical approach and is often categorized as low, medium, or high. High-fidelity simulation (HFS) is the use of full-scale electronic patient simulators, such as computerized mannequins that can mimic physiological responses, to provide realistic clinical symptoms and scenarios (Li et al., 2022). The immersive nature of this method is the key to its effectiveness. For instance, a study by Wojcieszek et al. (2025) concluded that high-fidelity simulation increases satisfaction and self efficacy among nursing students by combining attractive, hands-on scenarios with a debriefing process that emphasizes successful performance.

Unlike traditional clinical placements, simulation offers a safe, standardized environment where students can practice high-risk scenarios, make and reflect on their errors without risking patient safety, and receive immediate, structured debriefing. Through this process, simulation-based learning effectively facilitates experimental learning, which subsequently improves students' psychomotor skills and confidence (Nurfianti, 2023), with the debriefing phase being particularly crucial to this developmental process.

Despite its numerous advantages, the adoption of simulation-based learning is not without problems and constraints. As previously noted, the entire process is heavily dependent on the debriefing phase. This fosters reflective thinking through structured conversation and feedback following the simulation, enhances decision-making, and allows participants to organise their performance (Sahin & Basak, 2021), making the facilitator's abilities an integral point of potential failure. Hence, a poorly conducted debriefing led by an inadequately trained faculty can fail to consolidate learning or even reinforce incorrect practices.

Problem - Based Learning

Problem-based learning is a student-centered, outcome-based approach that has been proven to enhance the quality of learning across various disciplines and academic levels (Wei et al., 2023). In PBL, students and professors act as responsible partners in a process designed to build clinical knowledge, self-directed learning skills, and motivation to solve real-world problems (Jamshidi et al., 2021). This method substantially reorganizes the conventional classroom by utilizing real-world challenges to facilitate the learning process.

The hallmark of problem-based learning is the use of ill-structured problems, often a clinical case study with no clear solutions. Students, working in small collaborative groups first identify what they know, what they need to know, and where to find the necessary information (Matlala, 2021). A key figure in this process is the facilitator, who is not a traditional lecturer but a guide who asks probing questions,

encourages deeper inquiry, and helps maintain group dynamics without providing direct answers.

This structure encourages active problem-solving and collaboration among students, which are critical for deeper learning. Supporting this, a meta-analysis by Lu et al. (2025). found that PBL is significantly more effective than traditional instructional methods and enhanced evaluative and analytical skills. This is because the method requires learners to actively confront complex, real-world problems, forcing them to deeply analyze information, evaluate potential solutions, and justify their reasoning, which inherently fosters a deeper level of critical thinking.

Return Demonstration

Demonstrations and skills laboratories are a foundational pedagogical approach for teaching clinical skills, allowing students to first observe and then experience the realities of nursing (Msosa et al., 2022). The return demonstration, in particular, is a critical step that moves students from the role of passive observers to active participants. By physically performing a procedure, learners translate abstract concepts into tangible actions, a process that solidifies both muscle memory and procedural understanding (Ogunlowo & Ajibade, 2024). This active engagement provides an immediate opportunity for clinical instructors to assess and correct psychomotor skills, ensuring a foundational level of competence is established before students enter a live clinical setting.

The educational value of return demonstrations extends beyond mere technical proficiency. According to Raguini et al. (2020), return demonstrations allow nursing students to practice processes and assess learning outcomes, problem-solving skills, and reasoning abilities. Central to this entire process is the act of receiving immediate, constructive feedback from a clinical instructor (Burgess et al., 2020). This feedback loop is fundamental not only for minimizing the risk of reinforcing incorrect techniques but also for building student confidence and self-efficacy as they prepare for the complexities of patient care.

To ensure objectivity and fairness in assessment, instructors often use standardized checklists or rubrics that outline the critical steps of a procedure (Allen & Tanner, 2020), enabling consistent and evidence-based feedback. This structured approach facilitates the principle of "deliberate practice," where students don't just repeat a skill but focus on specific, targeted areas for improvement based on the feedback they receive (Nurse et al., 2024). It is this targeted cycle of performance, detailed feedback, and refinement that leads to true mastery and long-term skill retention. Ultimately, the use of laboratory-based return demonstrations significantly enhances the student's ability to apply theory to practice, with multiple studies confirming a marked improvement in their performance once they are exposed to the real clinical setting.

Clinical Exposure

Clinical exposure is indispensable for nursing students, providing the real-world context necessary to translate theoretical knowledge and laboratory skills into

professional practice. Research consistently highlights its multifaceted benefits. For instance, structured clinical placements significantly improve overall competence, with studies showing that students with extended exposure score higher in OSCE evaluations (Alharbi et al., 2024; Zhang et al., 2023). Beyond technical skills, interacting with real patients is crucial for enhancing adaptability and forging a strong professional identity (Li et al., 2022). This hands-on experience also develops essential soft skills, such as clinical decision-making under pressure, communication, and teamwork (Hamarash et al., 2025; Yoon et al., 2024).

The quality of the clinical environment itself is a critical factor. Positive and supportive settings, particularly those with structured mentorship from clinical instructors, have been shown to boost student confidence, performance, and skill transfer (Wei et al., 2023; Nurfianti et al., 2023; Brown & Singh, 2023; Durham, 2005). Furthermore, while simulation is a powerful tool, evidence suggests that integrating it with clinical exposure yields the highest levels of competence (Tong et al., 2024). As Cant and Cooper (2025) noted, the challenge lies in creating practice environments that effectively balance student learning needs with patient safety. Collectively, these findings underscore that direct clinical exposure is the cornerstone for consolidating knowledge into the well-rounded competence required of a nursing professional.

Peer Assisted Learning

Peer-assisted learning (PAL) is increasingly recognized as a valuable pedagogical approach in nursing education due to its wide-ranging benefits for student development.

The method has been shown to significantly enhance cognitive and psychomotor skills, with research demonstrating that PAL strengthens critical thinking, improves competence, and leads to better performance in skills assessments (Hamarash et al., 2025; Wood et al., 2025; Zhang et al., 2023). When combined with other methods like simulation, PAL can even lead to higher skill retention (Tong et al., 2024).

PAL yields substantial affective and social benefits. It creates a supportive environment that enhances motivation, boosts confidence, and reduces the anxiety often associated with skill demonstrations (Alharbi et al., 2024; Nurfianti et al., 2023). By its nature, PAL fosters a sense of belonging, teamwork, and collaborative reflection, while also improving student engagement during clinical training (Wood et al., 2025; Wei et al., 2023; Yoon et al., 2024). It also offers distinct pedagogical advantages, serving as a cost-effective supplement to traditional faculty-led instruction, particularly in resource-limited settings (Brown & Singh, 2023; Cant & Cooper, 2025). For senior students acting as tutors, it also provides an opportunity to develop essential leadership and teaching competencies, reinforcing their own professional identity (Durham, 2005; Hamarash et al., 2025).

Objective Structured Clinical Examination

The Objective Structured Clinical Examination (OSCE) is widely regarded as a gold standard for assessing clinical competence in nursing education (Cant & Cooper, 2025). Its primary strength lies in providing a reliable and standardized evaluation framework. By using structured scenarios and checklists, the OSCE minimizes examiner

bias and ensures that all students are assessed against the same objective criteria, making it a consistent performance assessment tool (Wei et al., 2023; Nurfianti & Mullen, 2024; Yoon et al., 2024). This structured approach allows for the objective evaluation of a wide range of skills, from psychomotor abilities to clinical decision-making (Li et al., 2022).

In addition to its reliability, the OSCE has excellent educational validity. Research has found that OSCE scores correlate strongly with overall clinical practice (Zhang et al., 2023; Brown & Singh, 2023). Its alignment with competence-based education models makes it an essential component for verifying that students have met required learning outcomes before graduation (Durham, 2005).

Furthermore, the OSCE serves a crucial formative function by directly contributing to student learning and development. It is a powerful diagnostic tool that clearly identifies individual strengths and weaknesses in skill mastery, allowing for targeted remediation (Alharbi et al., 2024). When followed by structured feedback, the OSCE has been shown to enhance reflective learning and, particularly when combined with peer feedback, can significantly improve student confidence (Tong et al., 2024; Hamarash et al., 2025). Collectively, the evidence overwhelmingly supports the OSCE as a valid, reliable, and indispensable assessment method in modern nursing education.

Local Studies

In the Philippine nursing education context, pedagogical approaches have continuously evolved to meet the demands of the healthcare system. Active learning strategies such as case-based discussions, peer mentoring, and skills demonstration

remain effective in improving nursing students' competence in local nursing schools. These approaches allow students to develop critical thinking while ensuring that practical competencies are aligned with the standards of the Commission on Higher Education (CHED) (Reyes, 2022).

The importance of faculty mentorship as a pedagogical approach has also been underscored. Rivera and Santos (2024) studied nursing schools in the Visayas and concluded that consistent guidance from clinical instructors enhanced both skill acquisition and professional behavior. Mentorship was found to provide emotional support, helping students navigate stressful clinical rotations while reinforcing technical competence. This aligns with global findings where structured instructor feedback is linked to improved student confidence and clinical readiness (Johnson & Lee, 2022).

Globally, pedagogical innovations have become central to advancing nursing education. Chen et al. (2022) in China demonstrated that combining simulation-based learning with reflective journaling significantly improved nursing students' psychomotor accuracy and decision-making in clinical practice. Their findings reinforce that modern pedagogical approaches bridge the gap between classroom knowledge and hospital-based application. In South Korea, Kim & Choi (2023) examined the effect of flipped classroom pedagogy on nursing students' clinical reasoning. Results showed that students who engaged in pre-class study and active class discussions demonstrated better clinical performance compared to those who followed traditional lecture methods. This confirms that student-centered pedagogies foster deeper engagement and skill mastery. United States, Smith et al. (2023) emphasized the role of relaxation techniques in clinical pedagogy. Their study found that students who practiced mindfulness and guided

breathing before clinical duties demonstrated reduced anxiety and greater accuracy in patient care procedures. This highlights the importance of integrating stress management strategies into teaching. Patel et al. (2023) in the United Kingdom reported that sleep-focused educational interventions improved psychomotor coordination and clinical reasoning among nursing students. Their study confirmed that adequate rest, when encouraged through structured pedagogy, significantly enhanced performance in OSCEs.

In Australia, Williams et al. (2022) studied the relationship between exercise programs and clinical endurance. Findings revealed that nursing students who maintained regular fitness routines showed better focus, reduced fatigue, and improved patient handling during extended clinical rotations. This points to the benefits of holistic pedagogy that addresses physical health.Canada, Johnson & Lee (2022) reported that peer-assisted learning groups enhanced both confidence and technical competence in clinical procedures such as catheter insertion and IV administration. Their findings affirm that peer support as a pedagogical tool provides a collaborative environment conducive to skill development. Meanwhile, in Vietnam, Nguyen et al. (2025) synthesized multiple international studies and concluded that curricula combining simulation, active learning, and wellness practices produced nursing students who were not only clinically competent but also emotionally resilient. Their review emphasized that pedagogical strategies should go beyond teaching techniques to include resilience-building approaches.In Saudi Arabia, Ahmed et al. (2025) investigated the effectiveness of holistic pedagogical frameworks in nursing schools. Results revealed that integrated strategies focusing on academic instruction, time management, and self-care produced students who demonstrated higher adaptability and stronger performance in clinical placements.

In Japan, Liu & Tan (2022) applied constructivist approaches in clinical skills training, highlighting that active participation and contextual learning significantly improved skill retention. Their findings suggest that pedagogical frameworks rooted in constructivism are effective across diverse educational settings. Finally, in Africa, Okeke & Mensah (2024) reported that blended learning approaches combining online resources and in-person laboratory practice improved the accuracy of nursing procedures and reduced student anxiety. This demonstrates that globally, integrating technology-enhanced pedagogy fosters stronger clinical outcomes.In the United Kingdom, Clarkson et al. (2023) highlighted the importance of scenario-based learning in nursing curricula. Students exposed to realistic patient case scenarios demonstrated improved problem-solving and communication skills, confirming that pedagogical strategies emphasizing clinical realism support competency development. In Canada,Morgan & Li (2022) reported that interprofessional education (IPE), where nursing students learned alongside medical and pharmacy students, improved collaboration skills and clinical confidence. This finding suggests that pedagogical approaches incorporating teamwork prepare nurses for real-world multidisciplinary environments.

In the Philippine context, nursing education continues to evolve in response to the challenges of healthcare delivery and the demands of competency-based curricula. Reyes and Dela Cruz (2022) reported that active learning methods, such as case-based instruction and peer learning, foster greater engagement and confidence among nursing students, leading to improved clinical performance. These findings highlight the effectiveness of pedagogical strategies tailored to Filipino learners. Closer to Digos City,

research has been conducted on professional nurses that indirectly informs the student context. A study on the quality of clinical care and communication skills of nurses in selected private hospitals in Digos City revealed that communication and skill proficiency were predictors of patient satisfaction (Scribd, 2023). While this focused on practicing nurses, it reflects the competencies expected of students in clinical placements, reinforcing the need for pedagogical approaches that strengthen communication, confidence, and patient interaction. Santos, K., & Perez, J. (2024). Additionally, Labrador (2025) explored the impact of peer-assisted learning on clinical skills acquisition among nursing students in Mindanao. Results revealed that peer mentoring sessions increased procedural accuracy and confidence, while simultaneously lowering anxiety levels during hospital rotations. This underscores the influence of peer support as a pedagogical tool in enhancing clinical performance. The importance of faculty mentorship as a pedagogical approach has also been underscored.

Rivera and Santos (2024) studied nursing schools in the Visayas and concluded that consistent guidance from clinical instructors enhanced both skill acquisition and professional behavior. Mentorship was found to provide emotional support, helping students navigate stressful clinical rotations while reinforcing technical competence. This aligns with global findings where structured instructor feedback is linked to improved student confidence and clinical readiness (Johnson & Lee, 2022).

Sleep and rest continue to surface as overlooked yet important pedagogical considerations. Torres (2023) found that Filipino nursing students with structured sleep patterns performed significantly better in objective structured clinical examinations (OSCEs). On the other hand, irregular sleep schedules were associated with decreased

alertness, lower return demonstration scores, and higher rates of clinical errors. These findings emphasize that educational programs must integrate lifestyle and wellness education alongside technical training. The literature indicates a shift toward a holistic approach to nursing education. Instead of focusing solely on psychomotor skills, pedagogy is increasingly addressing the cognitive, emotional, and physical well-being of students. Tan & Yu (2025) emphasized that curricula integrating relaxation, time management, peer support, sleep, and physical activity foster not only competence but also resilience and empathy—qualities essential for professional nursing practice. This holistic approach is highly relevant for institutions like PCDS, where the integration of lifestyle-based pedagogical strategies could yield significant improvements in students' clinical performance.

Pedagogical approaches serve as the foundation of effective nursing education, ensuring that students acquire not only theoretical knowledge but also practical clinical skills. Hernandez et al. (2023) emphasized that student-centered pedagogies, such as problem-based learning (PBL) and reflective discussions, improved the critical thinking and hands-on competence of nursing students during hospital duties. This demonstrates that modern teaching methods influence both cognitive and psychomotor domains of learning, which are essential in clinical practice**.** Delos Reyes and Bautista (2024) examined the effect of structured pedagogical interventions on the clinical competency of student nurses in Davao del Sur. Their findings indicated that strategies such as time management workshops, guided reflections, and simulation exercises improved clinical judgment, reduced procedural errors, and enhanced student confidence. This aligns with the study's focus on how varied pedagogical approaches influence clinical outcomes.The

role of active learning strategies has also been highlighted in the literature. According to Kim & Choi (2022), students exposed to active pedagogies, including simulation, role-playing, and team-based activities, displayed higher levels of clinical accuracy compared to peers taught through lecture-based methods. This suggests that the pedagogical approach itself is a determining factor in the quality of clinical skills nursing students develop.

In the Philippine context, Labrador (2025) explored the impact of peer-assisted learning on clinical skills acquisition among nursing students in Mindanao. Results revealed that peer mentoring sessions increased procedural accuracy and confidence, while simultaneously lowering anxiety levels during hospital rotations. This underscores the influence of peer support as a pedagogical tool in enhancing clinical performance. The development of clinical skills among nursing students is a cornerstone of nursing education, as these competencies determine their readiness to deliver safe, effective, and compassionate care in various healthcare settings. Recent literature highlights that pedagogical approaches play a vital role in shaping the clinical abilities of nursing students. Effective teaching strategies not only enhance technical proficiency but also build the students' capacity for critical thinking, empathy, and clinical judgment—qualities that are indispensable in nursing practice.Simulation-based learning has emerged as one of the most effective pedagogical approaches for nursing education. Studies reveal that the integration of role-playing, simulation, and traditional lectures leads to a significant improvement in nursing students' empathy, teamwork, and decision-making skills. This combination helps students experience real-world scenarios

in a safe and controlled environment, thereby increasing their confidence and readiness for clinical practice (PubMed, 2024).

Similarly, peer learning in simulation settings, supported by structured feedback and role observation, fosters collaboration and enhances skill acquisition, giving educators and students a common framework for clinical training. Work-integrated learning, or experiential learning in clinical settings, is another powerful pedagogical approach. Evidence shows that the effective application of theory into practice requires close supervision, structured learning opportunities, and supportive collaboration between faculty and clinical instructors (BMC Medical Education, 2019). These approaches enable students to develop a “sense of salience,” or the ability to prioritize and respond to complex patient situations, which is essential for professional practice. Problem-based learning (PBL) and peer-assisted strategies also contribute significantly to the enhancement of clinical skills. Research indicates that nursing students who are engaged in PBL develop stronger communication, problem-solving, and clinical reasoning skills (Sage Journals, 2021). By working collaboratively on clinical cases, students gain the ability to apply theoretical knowledge to practical problems, reinforcing both cognitive and psychomotor competencies.The clinical learning environment also plays a crucial role in shaping students’ learning outcomes.

A European study found that the atmosphere of clinical placements, the quality of supervision, and the level of pedagogical cooperation with nurse educators significantly influence students’ engagement, professional identity formation, and readiness for clinical practice (BMC Medical Education, 2022). In this regard, nurse educators must combine pedagogical expertise with clinical credibility to ensure effective and

trustworthy teaching. In addition, caring and cultural competence have been recognized as essential dimensions of nursing pedagogy. Pedagogical strategies such as immersive learning, service learning, and simulated interactive activities have been effective in cultivating caring behaviors and sensitivity to cultural diversity among nursing students (ScienceDirect, 2024). These approaches prepare students not only to deliver technical care but also to practice with compassion and respect for diverse patient populations. Evidence-based practice (EBP) integration into nursing education is also vital for strengthening clinical judgment.

Studies have shown that when EBP-oriented teaching is incorporated into the curriculum, students demonstrate greater confidence and competence in clinical decision-making (BMC Nursing, 2023). By linking research evidence to patient care scenarios, educators enable students to appreciate the value of evidence in guiding practice. Structured training courses focusing on clinical reasoning have also been found to improve students' knowledge and decision-making abilities, with positive feedback regarding their relevance to real-world nursing challenges (MDPI, 2024).Taken together, the literature suggests that a variety of pedagogical approaches—including simulation, peer learning, problem-based learning, evidence-based teaching, and supportive clinical environments—work synergistically to develop the clinical skills of nursing students. For institutions such as the Polytechnic College of Davao del Sur, adopting these strategies could significantly enhance the readiness of future nurses to meet the demands of healthcare practice. By aligning teaching methods with evidence-based strategies and fostering supportive clinical environments, nursing educators can ensure that students graduate not only with technical competence but also with the critical thinking, empathy,

and professional identity necessary to excel in their field.Sage Journals, 2021)Clinical placements are a core part of nursing education as they provide real-life learning opportunities. These experiences allow students to gain hands-on skills and apply theoretical knowledge. However, placements are becoming limited due to several challenges.

The increasing number of nursing students creates competition for clinical sites. Ward nurses also face staffing issues and heavy workloads that restrict student supervision. Patient safety and ethical concerns further reduce direct exposure to clinical practice. Because of these issues, pedagogical strategies like clinical simulation have become essential. Simulation creates safe and controlled learning environments for students. It uses real-life scenarios to strengthen knowledge and skills without risking patients. Various approaches such as high- and low-fidelity manikins, role play, and standardized patients are applied. Computer-based games also support the development of critical thinking and decision-making. These methods improve both technical and interpersonal clinical skills. Simulation ensures learning activities match students' needs and academic levels. In PCDS, such pedagogical approaches help bridge the gap in limited practicum experiences. Overall, clinical simulation strongly influences the enhancement of nursing students' clinical competence.

Theoretical Framework

The study entitled "The Relationship Between Pedagogical Approaches on Clinical Skills of Nursing students at Polytechnic College of Davao del Sur (PCDS)" is

anchored on a combination of educational and psychological learning theories that collectively explain how nursing students acquire and translate theoretical knowledge into practical clinical competence.

Constructivist Learning Theory

Constructivism, championed by theorists like Jean Piaget and Lev Vygotsky, posits that learners actively construct their own understanding and knowledge of the world through experiences and reflection. In nursing education, this theory strongly supports the use of student-centered pedagogical approaches such as Problem-Based Learning (PBL), Simulation, and Peer-Assisted Learning (PAL). These methods are particularly relevant to clinical skills development as they encourage students to move beyond passive reception of content to active engagement, critical thinking, and contextualized learning, thereby linking their current understanding to new practical scenarios.

Experimental Learning Theory

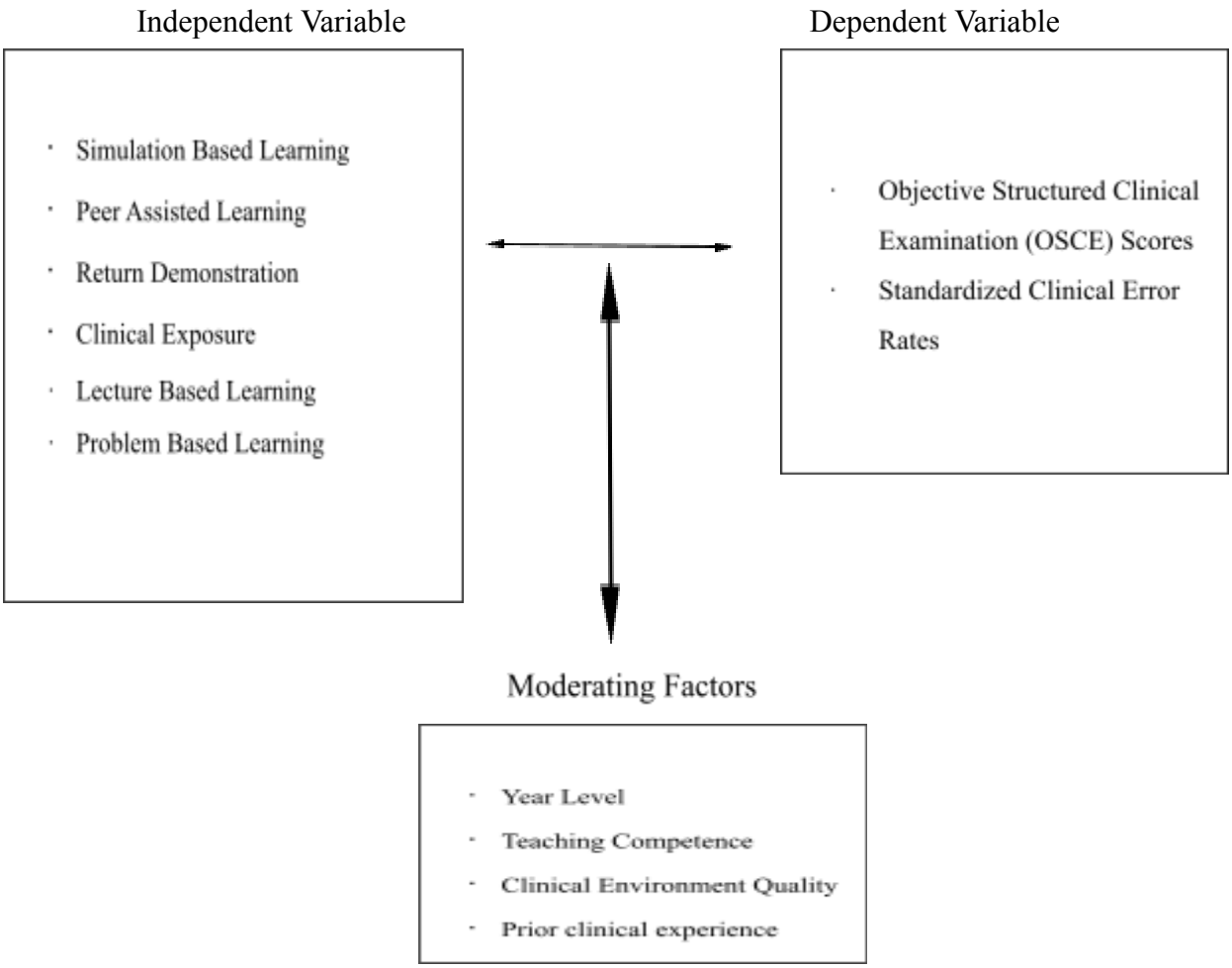
David Kolb's Experiential Learning Theory (ELT) emphasizes that knowledge is created through the transformation of experience. The learning process operates as a cycle involving four distinct stages: Concrete Experience, Reflective Observation, Abstract Conceptualization, and Active Experimentation. Clinical skills development

among nursing students is a direct application of this model. Return Demonstrations, clinical simulations, and supervised practicum align perfectly with the "Active Experimentation" and "Concrete Experience" stages, requiring students to practice skills, reflect on their performance, and adapt their techniques accordingly, thus contributing directly to improved psychomotor competency.

Benner's Novice to Expert Model

Patricia Benner's Novice to Expert Model outlines the developmental progression of nursing proficiency across five levels: Novice, Advanced Beginner, Competent, Proficient, and Expert. This framework provides a crucial developmental lens for clinical skills acquisition, recognizing that students move through these stages as they accumulate experience. The model supports the need for varied and progressively complex pedagogical strategies, justifying why the effectiveness of an approach may depend on the student's current level of expertise. Educators must tailor guidance, critical reflection, and scenario-based learning to facilitate this essential progression toward independent professional practice.

Conceptual Framework



This study is anchored on a correlational framework that illustrates the hypothesized relationship between the pedagogical approaches and the objective clinical skill outcomes of nursing students. The Independent Variables represent the core teaching strategies investigated, including Simulation-Based Learning, Peer-Assisted Learning, Return Demonstration, Clinical Exposure, Lecture-Based Learning, and Problem-Based Learning (PBL). The Dependent Variables consist of the students' measurable clinical skill outcomes: Objective Structured Clinical Examination (OSCE) scores and

standardized clinical error rates. This relationship is hypothesized to be influenced by Moderating Factors such as Year Level, Teaching Competence, Clinical Environment Quality, and Prior Clinical Experience as students in different phases of their program or those with varied prior exposure will likely respond differently to teaching strategies. This framework demonstrates that the study aims to determine the extent to which these pedagogical approaches influence clinical skills, while rigorously considering the impact of these context-specific student and institutional characteristics.

Statement of the Problem

The study aims to determine the extent of the relationship of pedagogical approaches on objective clinical skill outcomes among nursing students at Polytechnic College of Davao del Sur (PCDS).

1. What is the demographic profile of the participating nursing students in terms of
    - 1.1. Age
    - 1.2. Sex
    - 1.3. Year Level
2. What is the level of utilization of various pedagogical approaches in in terms of
    - 2.1. Simulation Based Learning
    - 2.2. Peer Assisted Learning
    - 2.3. Return Demonstration
    - 2.4. Clinical Exposure
    - 2.5. Lecture Based Learning

2.6. Problem Based Learning

3. What is the level of clinical skills among the nursing students in terms of:

3.1. Objective Structured Clinical Examination Scores

3.2. Standardized Clinical Error Rates

4. Is there a significant difference of the pedagogical approaches among the respondents in terms of:

4.1. Simulation Based Learning

4.2. Peer Assisted Learning

4.3. Return Demonstration

4.4. Clinical Exposure

4.5. Lecture Based Learning

4.6. Problem Based Learning

5. Is there a significant difference of clinical skills among the respondents in terms of:

5.1. Objective Structured Clinical Examinations Scores

5.2. Standardized Clinical Error Rates

6. Is there a significant relationship between the pedagogical approaches utilized and the students' objective clinical skill outcomes among nursing students at PCDS?

Statement of Hypothesis

H_0:There is no significant relationship between the pedagogical approaches on the objective clinical skill outcomes of nursing students

Significance of the Study

This correlational study holds significant empirical and contextual value for evidence-based practice and curriculum reform in resource-constrained settings. The findings are expected to benefit the following stakeholders:

Nursing Students: By identifying which specific teaching methods most strongly correlate with high objective performance, the findings empower students to maximize their learning experiences, engage more actively with proven strategies, and ultimately enhance their preparedness for safe and effective practice.

Nursing Faculty: The study provides context-specific empirical evidence on the effectiveness of strategies like low-fidelity demonstration, PAL, and clinical exposure in fostering measurable outcomes. This data enables faculty to refine instructional methods, justify the quality of their current practices, and select pedagogical approaches that demonstrably improve student competence.

Polytechnic College of Davao del Sur and Nursing Institutions: The research provides institutional leaders with evidence-based data crucial for curriculum design, resource allocation, and program enhancement. The findings will guide strategic investment in high-impact, cost-effective pedagogies and faculty development, thereby strengthening the institution's accreditation performance and ensuring the program remains responsive to local healthcare demands.

Healthcare Community: By improving the competence of nursing students, the study indirectly benefits patients and the broader healthcare system. Competent graduates entering clinical practice are better prepared to deliver safe, efficient, and high-quality

patient care. This contributes to improved health outcomes and reinforces public trust in healthcare professionals trained under pedagogically sound nursing programs.

Policymakers and Accrediting Bodies: The study offers data-driven recommendations that can inform and validate local and national policies on simulation usage and required standards for clinical competency. It supports the continuous improvement of instructional quality within nursing education frameworks.

Researchers. The study contributes to the growing body of literature on nursing education and clinical pedagogy, particularly in the Philippines. It identifies gaps in current teaching practices and offers empirical findings that can be used as a basis for comparative studies across different institutions. Researchers can also build on this study to explore moderating factors such as year level, faculty competence, or learning environment in relation to pedagogical effectiveness.

Future Researchers: This study contributes to the literature on clinical pedagogy, specifically filling the gap concerning correlational data in low-resource Philippine settings. It provides empirical findings and a research model that future scholars can use as a foundation for comparative studies across different institutions or for further investigation into the influence of specific moderating factors on pedagogical effectiveness.

Scope and Delimitation of the Study

The scope of this correlational study is narrowly focused on examining the relationship and extent of association between specific pedagogical approaches and the

objective clinical skill outcomes of nursing students at the Polytechnic College of Davao del Sur (PCDS). The study population includes all enrolled students from the 1st year to 4th year levels in the Bachelor of Science in Nursing program during the academic year 2025–2026. The pedagogical approaches investigated as independent variables include Lecture-Based Learning, Simulation-Based Learning, Problem-Based Learning, Return Demonstration, Clinical Exposure, and Peer-Assisted Learning (PAL), while the clinical skills are strictly measured by the students' Objective Structured Clinical Examination (OSCE) scores and standardized clinical error rates, retrieved from institutional records. The study is strictly delimited by its correlational design. It can determine the strength and direction of the relationships but cannot establish direct cause-and-effect between a specific teaching method and outcomes; furthermore, the findings are primarily applicable to the context of PCDS and institutions with similar resource constraints, as the measurement of clinical skills is limited to objective, summative institutional data and excludes qualitative or long-term clinical competency evaluations.

Definition of Terms

In this section, the important terminologies are defined based on their operational meaning within the context of this study to ensure thorough understanding.

Pedagogical Approaches: These refer to the instructional strategies and methods utilized by educators to enhance clinical learning outcomes among nursing students at PCDS. For the purpose of this study, these independent variables include Lecture-Based Learning, Simulation-Based Learning, Problem-Based Learning, Peer-Assisted Learning

(PAL), Return Demonstration, and Clinical Exposure. The relationship of these approaches will be quantified by the weighted mean scores derived from the self-structured Likert scale survey questionnaire administered to the participants.

Objective Structured Clinical Examination: This is the primary dependent variable used to measure students' clinical competence. It is a standardized assessment method where students perform specific clinical tasks at rotating stations, demonstrating their psychomotor skills and decision-making abilities. In this study, OSCE is operationally defined as the raw numerical scores obtained by the participants, which are retrieved from authenticated institutional records.

Standardized Clinical Error Rates: This is the secondary dependent variable utilized for objective evaluation of clinical skill outcomes. It refers to the frequency of critical errors observed and recorded on standardized institutional checklists during summative skill assessments and supervised clinical exposures. This data is retrieved from authenticated school records and serves as a quantifiable measure of clinical performance.

Clinical Skill Outcomes: This term collectively refers to the students' objective dependent variables in this study, which are quantitatively measured by their Objective Structured Clinical Examination (OSCE) Scores and Standardized Clinical Error Rates.

Teaching Competence: Operationally defined as the students' perception of their instructors' teaching effectiveness. This variable will be measured quantitatively as the mean score derived from student responses to a 4-point Likert scale on the self-structured

survey. Survey items for this variable will include questions that assess the clarity of instruction, quality of feedback, and facilitation skills.

Clinical Environment Quality: Operationally defined as the students' perception of the learning environment in their clinical placements and skill laboratories. This variable will be measured as the mean score derived from a 4-point Likert scale on the self-structured survey. Items will assess factors such as the supportive nature of the clinical staff, the availability of resources, and the overall atmosphere for learning and practice

CHAPTER II

METHODOLOGY

This chapter presents the rigorous methods and procedures used to conduct the study and ensure the validity of findings regarding the influence of pedagogical approaches on clinical skills. It includes the research design, respondents, sampling technique, research locale, research instrument, data gathering procedure, statistical treatment, and ethical considerations.

Research Design

This study will employ a correlational research design to explore the relationship between various pedagogical approaches and the objective clinical skill outcomes of nursing students. A correlational design is appropriate as it allows the researchers to examine the degree and direction of association between variables without manipulating them, thereby offering insights into naturally occurring relationships within an academic setting (Creswell, 2018). The independent variables in this study will include different pedagogical approaches, such as lecture-based learning, simulation-based learning, problem-based learning, return demonstration, clinical exposure, and peer-assisted learning. The dependent variables will be the students' objective clinical outcomes, specifically their Objective Structured Clinical Examination (OSCE) scores and standardized clinical error rates. This design enables the researchers to assess the strength and direction of the relationship between teaching strategies and skill development, which is essential for advancing evidence-based practices in nursing education.

Ethical considerations are paramount in this study to ensure the rights and well-being of participants. Ethical approval will be sought from the institutional review board (IRB) to ensure adherence to ethical standards. Informed consent will be obtained from all participants, with comprehensive information provided about the nature of the study, potential risks, and their right to withdraw without consequence. Participants' confidentiality will be strictly maintained, with personal information de-identified and securely stored. Data will be presented in aggregate form to prevent identification of individuals. To ensure the study causes no harm, the principle of non-maleficence will guide the research, offering appropriate support to participants should they experience any distress during clinical assessments. Additionally, the principle of beneficence ensures that the study will aim to benefit nursing students by enhancing teaching practices that foster clinical skills development, ultimately improving patient care. Participation will be entirely voluntary, with students able to withdraw from the study at any time without affecting their academic standing. Finally, the study will strive to maintain equity by ensuring diverse representation of students from various backgrounds and learning needs, thereby avoiding discrimination or exclusion. These ethical principles will ensure that the research contributes to the field of nursing education while upholding the dignity and rights of all participants Creswell, J. W. (2018).

Respondents of the Study

The target population (N) for this correlational study consists of approximately 1,400 nursing students enrolled across the first-year to fourth-year levels in the Bachelor of Science in Nursing program at the Polytechnic College of Davao del Sur during the academic year 2025–2026. This population is selected because the students' progression across the four year levels represents different stages of academic and clinical exposure, providing a holistic perspective on pedagogical effectiveness and aligning with the developmental principles of Benner's Novice to Expert Model. To ensure the data's statistical reliability and power for subsequent regression analysis, the final sample size (n) was calculated to be 100 respondents. This robust sample will be drawn using a Stratified Random Sampling technique with equal allocation to ensure balanced representation across the four key stages of skill acquisition, yielding 25 respondents from year levels. Inclusion criteria were students currently enrolled who provided written informed consent and whose objective clinical records were accessible, while exclusion criteria involved students on a leave of absence or those who declined participation.

Sampling Technique

This study will employ a stratified random sampling method to ensure a representative and developmentally diverse selection of nursing students enrolled at the Polytechnic College of Davao del Sur (PCDS). This technique is deemed most appropriate because nursing students across year levels possess varying degrees of clinical exposure and academic preparation, which serves as a critical moderating factor

in clinical performance. By treating each year level (Level I–IV) as a distinct stratum, the research ensures a balanced analysis of how varied pedagogical strategies impact clinical outcomes across the stages of professional development. The sample size was calculated based on the total population (N≈1,400) using standard statistical methods to achieve a final target sample of 100 students (n=100) at a 95% confidence level and 5% margin of error. Equal allocation was applied to determine the number of participants to be drawn from each stratum based on the enrollment size of that year level. Within each stratum, participants will be randomly selected using a random number generator ensuring that every eligible student will have an equal probability of inclusion. Selected respondents will be contacted and provided with complete research information, including confidentiality measures and ethical assurances. Participation will be voluntary, and only students who provide signed informed consent will be included in the study.

Research Locale

The researchers will be conducting the study at Polytechnic College of Davao del Sur Inc, situated along Mcarthur Highway in Kiagot, Digos City, Davao del Sur, Philippines. The college offers a comprehensive nursing program designed to equip students with the essential skills and knowledge needed for effective clinical practice.

Site Map

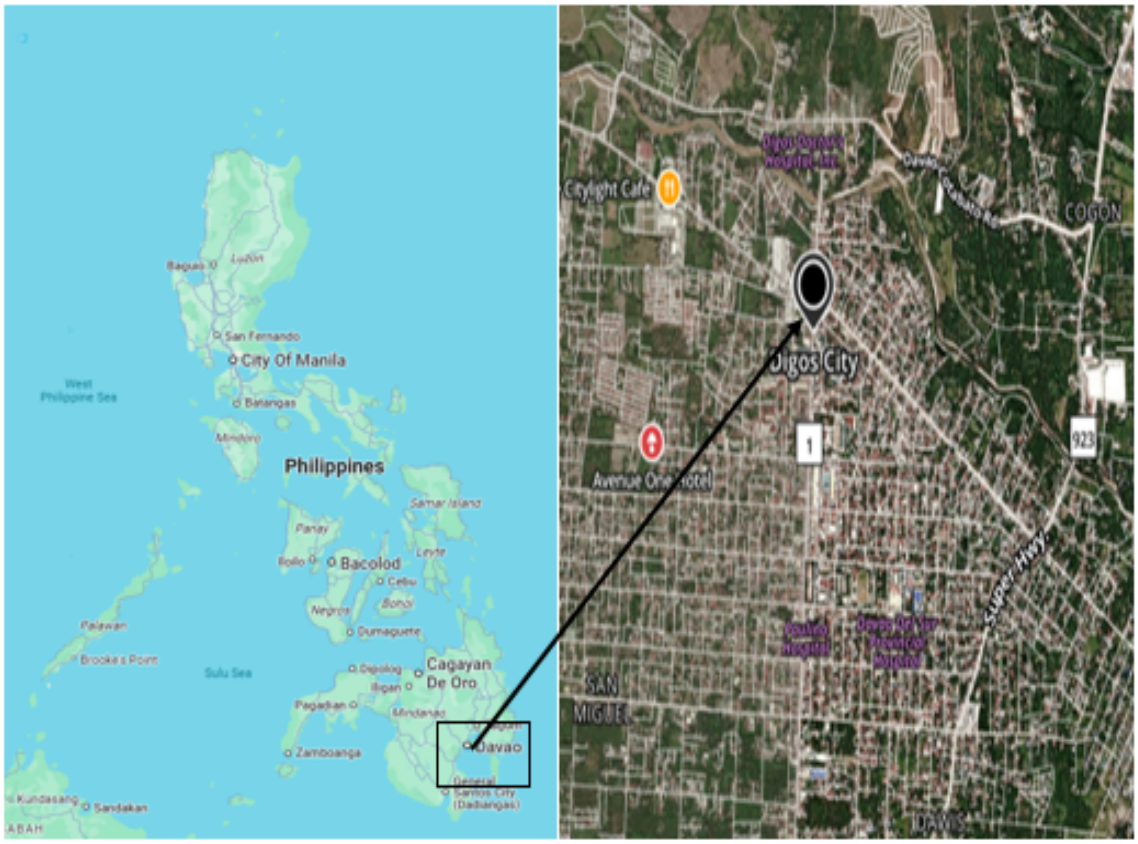


Figure 2: *Site Map*

Research Instrument

The research methodology employs a two-pronged data collection approach aligned with the study's correlational design, utilizing both a self-structured survey questionnaire and objective institutional records.

The primary component is a self-structured survey questionnaire developed by the researchers, which is divided into two parts. The first part focuses on the demographic and background profile of respondents (e.g., age, sex, year level, and prior clinical experience). The second part contains items designed to assess students' perceived exposure to and the relationship of various pedagogical approaches on their clinical skill development. A 4-point Likert scale is used to quantify responses in this section (where 4= strongly agree to 1= strongly disagree). These responses will be interpreted using weighted mean scores and treated as interval data for correlation and regression analysis. The instrument will undergo rigorous content and face validation by three experts in nursing education and two subject matter experts to ensure clarity and direct alignment with study objectives. A pilot test will be subsequently conducted with a group of 20 nursing students not included in the final sample to perform a reliability analysis and ensure acceptable internal consistency.

The second, and crucial, component of the instrument involves accessing authenticated institutional records to obtain the objective dependent variables. This data specifically includes Objective Structured Clinical Examination (OSCE) scores and standardized clinical error checklist data for each participating student. Prior to all data collection, an informed consent form will be provided to the respondents, ensuring voluntary and ethical participation and explicit consent for the use of their school records.

Likert Scale

NUMERICAL EQUIVALENT	RANGE	DESCRIPTIVE EQUIVALENT	VERBAL INTERPRETATION
4	3.25 – 4.00	STRONGLY AGREE	I fully agree with the statement. / Buo akong gisuportahan ang maong pamahayag.
3	2.50 – 3.24	AGREE	I agree with the statement. / Uyon ako sa maong pamahayag.
2	1.75 – 2.49	DISAGREE	I disagree with the statement. / Supak ako sa maong pamahayag.
1	1.00 – 1.74	STRONGLY DISAGREE	I strongly disagree with the statement. / Kusgan kong supak sa maong pamahayag.

Data Gathering Procedure

The data collection process is strategically designed to enable the correlational evaluation of the influence of pedagogical approaches on clinical skills among nursing students at Polytechnic College of Davao del Sur. The process entailed institutional approvals, participant recruitment using a rigorous sampling technique, self-report survey administration, and the retrieval of objective performance records. The following steps will be employed:

1. The researchers will first prepare a formal letter requesting approval to conduct the study. This letter, duly endorsed by the research adviser, will be submitted to the Dean of the Nursing Department and the College Registrar of Polytechnic College of Davao del Sur, Inc. Approval will be granted after the review of the study's objectives, research questions, and methodology.
2. After approval, the researchers will employ a stratified random sampling technique with proportional allocation to ensure all year levels of nursing students are fairly represented. Stratification will be based on year level to account for

differences in clinical exposure and learning experiences. A random number generator will then be used to select the participants from each stratum, resulting in a target sample size of n=100.

3. The selected students will be provided with an informed consent form before participation. The form will outline the purpose of the study, the procedures involved, the voluntary nature of participation, and assurances of confidentiality and anonymity. Crucially, the form will also explicitly request consent for the researchers to access their Objective Structured Clinical Examination (OSCE) scores and final clinical grades from institutional records. Only those who sign the informed consent will be included in the study.
4. The researchers will distribute the self-made survey questionnaire. This instrument consists of two parts: (a) demographic profile (age, sex, year level, prior clinical experience), and (b) items measuring the perceived influence of pedagogical approaches on clinical performance. Data will be collected both face-to-face (during students' free time in classrooms) and via secure online platforms to maximize participation. The researchers will be present during face-to-face administration to provide clarification and ensure proper completion.
5. Utilizing the consent obtained in Step 3, the researchers will submit a formal request to the College Registrar and the Dean of the Nursing Department to retrieve the objective performance data for the consented participants. This data will include Objective Structured Clinical Examination (OSCE) scores, standardized clinical error checklist data, and final clinical grades. This

institutional data will be securely handled and linked to the survey responses using a unique, non-identifying code.

6. All responses and retrieved records will be treated with strict confidentiality and anonymity. Completed questionnaires and performance data will be coded numerically, with no identifying information recorded. All raw data will be secured in password-protected files accessible only to the researchers and the research adviser, thus adhering to the highest ethical standards.

Statistical Treatment of Data

The collected data will undergo analysis using the following statistical tools, each aligned with the study's specific objectives:

Frequency and Percentage. These will be used to present the demographic profile of the respondents, such as age, sex, and year level. This will help in tabulating categorical data to identify the distribution of respondents across different demographic groups.

Mean. The mean will be employed to ascertain the average responses of students regarding the influence of various pedagogical approaches on their clinical performance and clinical grades. This will reveal the general trend of agreement among the respondents

T-Test. An independent sample t-test will be employed to identify if there is a significant difference in the perceived effectiveness of pedagogical approaches on clinical

skills when grouped according to sex. This statistical tool is appropriate for comparing the means between two independent groups.

ANOVA (Analysis of Variance). ANOVA will be used to determine whether there is a significant difference in the influence of pedagogical approaches on clinical skills across the four year levels of nursing students. This test will allow the researchers to examine if the effectiveness of teaching strategies varies depending on the students' academic level.

Pearson Correlation. Pearson correlation will be employed to determine the strength and direction related between pedagogical approaches and the clinical skills of nursing students. This test will establish whether increased exposure to specific pedagogical methods is associated with higher clinical performance and grades. Statistical significance will be determined at a 0.05 level of significance.

Ethical Considerations

Ethical considerations remained paramount throughout the research process to ensure that participants' rights and academic well-being were adequately protected. Informed consent will be sought from all participating nursing students, who were fully informed regarding the purpose of the study, and the minimal risks or potential benefits. They will be also informed of their right to withdraw from the study at any time without any negative consequences to their academic standing or relationship with the college. This ensures that participation is entirely voluntary and that the students' welfare is respected.

Confidentiality will be ensured. All data obtained from the participants, including the survey responses and the objective performance data retrieved from institutional records, will be anonymized. No personal identifiers will be attached to their responses or records; instead, a unique code will be used to link the data sets. The data will be kept in a secure, password-protected location and only accessible to research personnel authorized to access it. All records will be safeguarded and treated as confidential during the research period and after the completion of the study to ensure privacy.

The study followed the ethical standards set by relevant institutional review boards (IRB) and complied with all applicable laws and regulations. This includes ensuring that the study is conducted with integrity and transparency, minimizing any potential harm to participants, and maintaining the highest standards of ethical research practices. Formal institutional approval to conduct the study and access the necessary administrative data (OSCE scores, grades) will be secured from the College administration.

In addition, the research avoided plagiarism since proper acknowledgment of all sources of information was ensured, and all data and materials used were properly cited. Proper citation of all external resources used in the development of the research is done. The research is expected to benefit the community by providing valuable insights into the factors influencing the clinical skills development of nursing students in a local, resource-constrained setting like the Polytechnic College of Davao del Sur. The findings have the potential to inform local curriculum reviews, faculty development programs, and pedagogical interventions aimed at improving objective clinical competence. Participants also benefited from the study by gaining awareness of the importance of various teaching

strategies and the factors that contribute to their clinical proficiency, which may lead to positive changes in their self-directed learning and overall academic well-being.

To sum it up, this study upheld the highest ethical standards by ensuring informed consent, confidentiality, academic integrity, and a focus on the potential benefits for the institution and the participants' professional development.